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Where Neuroscience Meets Ancient Wisdom

PMDD: A Non-Pharmacological Action Plan

Premenstrual Dysphoric Disorder: lifestyle and behavioural strategies you can start this cycle

PMDD is a cyclical, biologically driven sensitivity to normal hormonal shifts in the luteal phase, not a lack of willpower. The measures below are first-line for mild-to-moderate symptoms and remain essential alongside medication for moderate-to-severe PMDD. Apply them consistently across the cycle, not only when symptoms appear.

Start here: track before you treat

Rate mood, anxiety, irritability, sleep, energy and physical symptoms daily for 2 cycles (0–3 severity scale, or a DRSP-style diary). This confirms the luteal-phase pattern and identifies your personal high-risk window, usually the 6 days before menses.

Four daily habits that change the course of PMDD

01

MOVE: AEROBIC EXERCISE

- 3–5 sessions/week, 30–45 minutes, moderate intensity (brisk walk, cycling, swimming)
- Keep sessions going through the luteal phase, lighter is fine, but do not skip
- Yoga or Pilates 2–3x/week adds both movement and stress-reduction benefit

02

SLEEP: FIXED SCHEDULE

- Same bed and wake time (± 30 min) every day, weekends included
- 7–9 hours nightly; no screens for 60 min before bed
- Caffeine cut-off by 2 pm; avoid alcohol as a sleep aid

03

EAT: STABLE-BLOOD-SUGAR PLATE

- Half vegetables, quarter protein, quarter complex carbs (oats, millets, whole wheat) each meal
- Small, frequent meals; pair carbs with protein to blunt cravings and mood dips
- Reduce salt (bloating), caffeine (anxiety), alcohol and ultra-processed food, especially pre-menses

04

REGULATE: DAILY STRESS PRACTICE

- 10 minutes/day of breathing work (e.g. 4-7-8 breathing) or mindfulness
- Progressive muscle relaxation 10–15 min/day in the luteal phase
- One longer practice/week: guided yoga or an MBSR-style session





Match the strategy to the dominant symptom

Dominant symptom	Prioritise
Anxiety / irritability	CBT, mindfulness, yoga, daily breathing practice
Bloating / physical pain	Low-salt diet, gentle exercise, heat, magnesium-rich foods
Cravings / mood swings	Regular small meals, complex carbs, calcium-rich foods

Supplements (adjunct, discuss with your clinician): calcium, vitamin D, magnesium and omega-3 have the best supporting evidence and are reasonable adjuncts once the four habits above are in place.

When to seek more help

If lifestyle measures do not meaningfully improve symptoms after 2–3 tracked cycles, if symptoms significantly impair work, relationships or daily functioning, or if there is any thought of self-harm or suicide at any point in the cycle, seek clinical review promptly.

There is no need to fear medication if it becomes necessary. PMDD is usually managed with intermittent, luteal-phase-only dosing rather than a daily medicine, which keeps the side-effect burden very low. Medication is a low-risk addition to the habits above, not a replacement for them.

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